

SKIN SYMPTOMS

Give urgent attention to the patient with skin symptoms and any of:

- If sudden generalised itch/rash or face/tongue swelling and any of: wheeze, difficulty breathing, BP < 90/60, dizziness/collapse, abdominal pain, vomiting or exposure to possible allergen¹, check for anaphylaxis ↗ 20.
- Purple/red rash with any of: neck stiffness, drowsy/confused, temperature ≥ 38°C, headache: **meningococcal disease** likely
- Diffuse rash appearing within 3 months of starting a new medication *and* any of the following, **serious drug reaction** likely:
 - BP < 90/60
 - Temperature ≥ 38°C
 - Abdominal pain
 - Vomiting or diarrhoea
 - Involves mouth, eyes or genitals
 - Blisters, peeling or raw areas
 - Jaundice



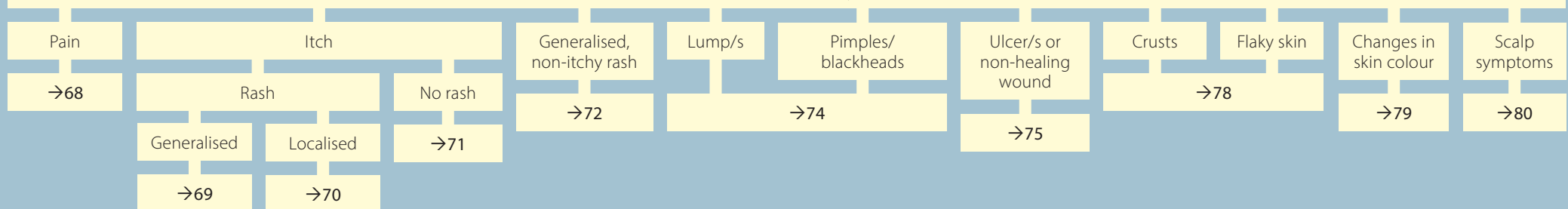
© University of Cape Town

Management:

- If **meningococcal disease** likely: give **ceftriaxone** 2g IV²/IM. Avoid injecting > 1g IM at one injection site.
 - Prevent disease in close household contacts:
 - If pregnant or child contact < 6 years old, give **ceftriaxone** 250mg IM.
 - If child 6-12 years old, give **ciprofloxacin** 250mg as a single dose.
 - If ≥ 12 years, give **ciprofloxacin** 500mg as a single dose.
- If **serious drug reaction** likely: stop all medication. If peeling or raw skin, also manage as for burns before referral ↗ 21.
- If BP < 90/60, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- Refer urgently.

Approach to the patient with skin symptoms not needing urgent attention

Manage according to skin symptom/s:



If rash is extensive, recurrent or difficult to treat, test for HIV ↗ 110.

¹Common allergens include medication, food or insect bite/sting within the past few hours. ²Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone.